SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Enzalutamide: castration-resistant prostate cancer

Metastatic or high-risk nonmetastatic castration-resistant prostate cancer · NCCP 00233 · version 7 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to control prostate cancer and delay progression. The expected benefit depends on your disease setting, previous treatment and general health.

Schedule
Enzalutamide 160 mg once daily, continuously while helping and tolerated. Continue ADT unless surgically castrated. A lower dose may be prescribed for toxicity or interactions.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Enzalutamide

ANDROGEN PATHWAY TREATMENT

- ☐ Fatigue, hot flushes, reduced concentration or memory problems
- ☐ Falls, fractures and loss of bone strength alongside ADT
- ☐ High blood pressure and ischaemic heart disease
- ☐ Rare seizure or posterior reversible encephalopathy syndrome with confusion, headache or vision changes
- ☐ Medicine interactions, including effects on anticoagulants
- ☐ Rash, swallowing difficulty, allergic reactions and sexual or fertility effects

Discuss the treatment aim, setting-specific evidence, alternatives and declining treatment; oral medicine and ADT risks, bone and cardiovascular health, sexual function, fertility, contraception, interactions, required tests and urgent contacts. Report falls, new memory or concentration problems and troublesome fatigue. Seizure, severe headache with confusion or vision changes needs urgent help. Discuss driving safety and every new medicine with the team; interactions can change anticoagulant effects.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00233 v7.

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